

Patient Referral to ATU, MCOT, Psychiatric ED

Standard:

To establish guidelines for referring individuals to the ATU, MCOT, Psychiatric ED.

Purpose:

To establish criteria for ATCEMS referral of persons to an approved alternate mental health resource in order to facilitate more appropriate evaluation and care.

Application:

General Applicability:

1. Does not require stretcher for safe and comfortable transport.
2. Does not require special precautions for infectious diseases.
3. Patient does not meet any alert criteria.
4. Will not require monitoring, re-evaluation of treatment or ongoing treatment during transport.

Immediate Exclusion Criteria:

If the patient meets any exclusion criterion, then the patient must be transported to an Adult or Pediatric ED per COG transport requirements.

1. Cannot sit, stand, walk, and pivot.
2. Suicide attempt within previous 24 hours
3. Any patient with ongoing bleeding, wounds requiring repair, or suspected head injury.
4. Any acute neuro-focal changes.
5. GCS < 14, Seizure within < 24 hours
6. Acute hypotensive syncope, or near syncope event(s)
7. Hypoxic-like event
8. Complaint of chest pain or shortness of breath
9. Has been and/or is expected to be violent
10. Evidence of GI bleeding
11. Female of childbearing age with any of the following:
 - a. Localized abdominal pain
 - b. LMP $\geq$ 12 weeks ago
 - c. Unusual or unexpected vaginal bleeding or discharge
12. Patient ingested medication, prescription or over the counter, outside of normal dosing range within the last 48 hours.

Vital Sign Requirements:

1. Pulse: 60 – 110 bpm
2. Systolic BP: 90 – 200 mmHg
3. Respirations: 12 – 30 bpm
4. Blood Glucose: 70 – 300 mg/dL; no signs of DKA
5. SPO₂ $\geq$ 93% on room air or prescribed supplemental oxygen

Circumstantial Evaluation, and all of these must apply:

1. Patient understands and follows commands.
2. Patient does not require physical restraint(s).
3. Patient is not in the custody of a peace officer.
4. A physician has not specifically requested ambulance transport.

Regardless of known history of hypertension, if SBP > 160 and/or DBP > 100 then the patient should be advised to seek follow up evaluation for their hypertension by community physician.

Minor superficial abrasions may be evaluated for underlying injury and dressed as needed by EMS.

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If the individual meets General Applicability, Vital Sign Requirements and Circumstantial Evaluation, but none of the Immediate Exclusion Criteria, then the patient may be referred to ATU, MCOT, or transported to psychiatric ED

If tele-psych services are used, tele-psych provider will speak with EMS personnel and inform them of the plan that has been developed with and for the patient as well as tele-psych disposition. If patient ends the call, field providers should expect tele-psych provider to contact them. As with all telehealth interactions, field providers must document this in addition to standard patient care and narrative.